SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nab-paclitaxel · every 3 weeks

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Metastatic breast cancer after first-line failure; standard anthracycline therapy unsuitable · NCCP 00230 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control advanced cancer and symptoms while balancing side effects and treatment burden.

Schedule
Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nab-paclitaxel
CHEMOTHERAPY

- ☐ Numbness / tingling or weakness, sometimes lasting
- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, fatigue and muscle / joint aches
- ☐ Infusion reactions; uncommon serious lung inflammation

Discuss the exact setting, expected benefit, alternatives, medicine-specific risks, monitoring, contraception and urgent contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00230 v7.

SACTCheck v0.84.19
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